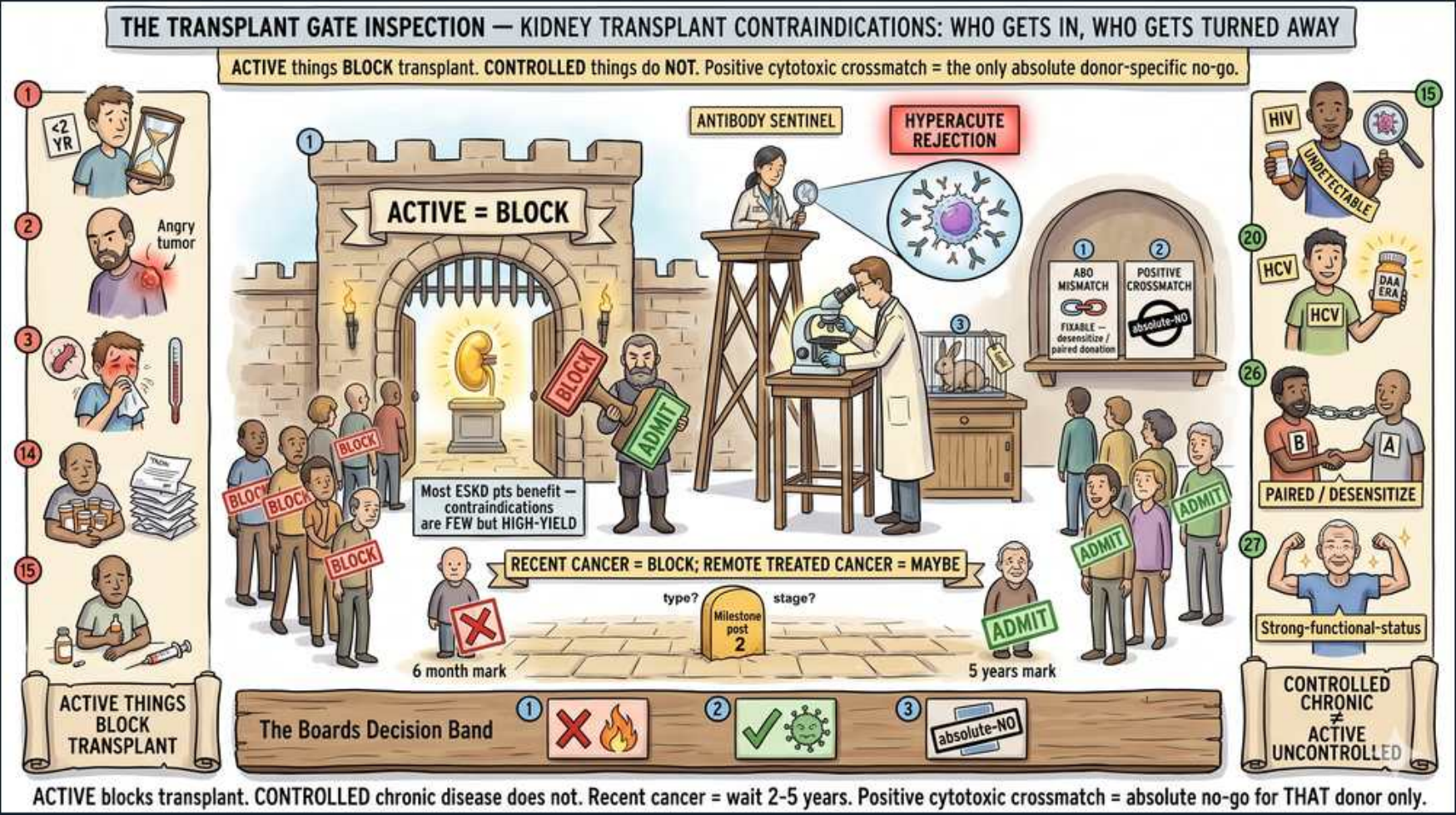


Kidney Transplant Contraindications — Who Gets In, Who Gets Turned Away



ACTIVE blocks transplant. CONTROLLED chronic disease does not. Recent cancer = wait 2-5 years. Positive cytotoxic crossmatch = absolute no-go for THAT donor only.

1. ACTIVE = block (the gate rule)

WHAT YOU SEE

Gate banner: ACTIVE = BLOCK

WHAT IT ENCODES

Core principle: ACTIVE problems (active infection, active malignancy, active substance abuse, untreated/active severe psychiatric or non-adherence) are contraindications. Once controlled, most are no longer absolute barriers.

BOARD TRAP

Most contraindications are relative once disease is CONTROLLED — 'has cancer history' alone is not an absolute no-go.

2. Active infection — blocks transplant

WHAT YOU SEE

BLOCK sign over active infection figure

WHAT IT ENCODES

Untreated active infection (e.g., active TB, untreated sepsis, active hepatitis with viremia) must be controlled before transplant, since immunosuppression will worsen it. Treat and clear first.

BOARD TRAP

Active infection delays, not permanently bars, transplant — contrast with positive cytotoxic crossmatch, which is an absolute donor-specific no-go.

3. Active malignancy — blocks; treated cancer maybe

WHAT YOU SEE

BLOCK over tumor, ADMIT after treatment

WHAT IT ENCODES

Active cancer is a contraindication. After curative treatment, a waiting period (commonly ~2–5 years depending on tumor type/stage) is observed before listing because immunosuppression promotes recurrence.

BOARD TRAP

Remote, treated cancer is often acceptable — it is not an absolute lifelong contraindication; the answer is 'wait, then list.'

4. Recent cancer → wait 2–5 years

WHAT YOU SEE

Milestone markers: 6-month vs 5-year

WHAT IT ENCODES

Wait time after cancer cure depends on type and stage — low-risk skin/in-situ may need little wait, while aggressive solid tumors need years of disease-free interval before transplant.

BOARD TRAP

There is no single universal wait — high-stage aggressive tumors need a longer disease-free interval than a treated low-grade lesion.

5. Hyperacute rejection — preformed antibody

WHAT YOU SEE

Hyperacute rejection burst, antibody sentinel

WHAT IT ENCODES

Hyperacute rejection occurs within minutes–hours due to PREFORMED donor-specific (often anti-ABO or anti-HLA) antibodies that fix complement and thrombose the graft. It is prevented by crossmatching, not treated.

BOARD TRAP

Hyperacute rejection is immediate and untreatable — distinguish from acute cellular rejection (days–weeks, T-cell, steroid-responsive).

6. Positive cytotoxic crossmatch — absolute no-go

WHAT YOU SEE

ABO mismatch / positive crossmatch panel

WHAT IT ENCODES

A positive complement-dependent cytotoxicity crossmatch means preformed anti-donor antibody — an ABSOLUTE contraindication to THAT specific donor (predicts hyperacute rejection). It is donor-specific, not a universal bar.

BOARD TRAP

Positive crossmatch blocks only THAT donor pairing — the patient can still receive a different compatible kidney.

7. ABO mismatch

WHAT YOU SEE

ABO mismatch label in sentinel panel

WHAT IT ENCODES

ABO incompatibility risks hyperacute rejection but can be overcome with desensitization (plasmapheresis, IVIG, rituximab) or paired donation, unlike an unmodifiable positive crossmatch.

BOARD TRAP

ABO incompatibility is not absolute — desensitization or paired exchange can enable transplant; positive cytotoxic crossmatch is the harder barrier.

8. Paired donation / desensitization

WHAT YOU SEE

Paired/desensitize panel on right tower

WHAT IT ENCODES

For sensitized or ABO-incompatible pairs, options include kidney paired exchange (swap donors) and desensitization protocols (plasmapheresis + IVIG ± rituximab) to lower antibody before transplant.

BOARD TRAP

Sensitization is a barrier to be engineered around (paired exchange/desensitize), not an automatic disqualification from transplant entirely.

9. HIV — controlled is acceptable

WHAT YOU SEE

HIV bottle on right shelf

WHAT IT ENCODES

Well-controlled HIV (suppressed viral load, adequate CD4, on ART) is NOT a contraindication; HIV+ to HIV+ transplantation is performed. Uncontrolled/AIDS-defining illness is the barrier.

BOARD TRAP

HIV itself is no longer an absolute contraindication — controlled HIV with suppressed VL can be transplanted.

10. Hepatitis C — treatable, transplantable

WHAT YOU SEE

HCV bottle on right shelf

WHAT IT ENCODES

With direct-acting antivirals, HCV is curable; HCV+ donors can even be used for recipients with planned DAA treatment. Active untreated cirrhosis/decompensation is the real concern.

BOARD TRAP

HCV is no longer a bar — DAAs cure it, and HCV+ organs are deliberately used; don't reflexively exclude HCV recipients.

11. Controlled chronic disease — ADMIT

WHAT YOU SEE

ADMIT figures, strong functional status

WHAT IT ENCODES

Stable, controlled chronic conditions (well-managed diabetes, controlled HTN, compensated cardiac disease) and good functional status favor candidacy. Most ESRD patients benefit from transplant over dialysis.

BOARD TRAP

Controlled comorbidity does NOT block transplant — most candidates benefit; few contraindications are absolute but high-yield.

12. Active substance abuse / non-adherence

WHAT YOU SEE

BLOCK stack of papers, active-things-block panel

WHAT IT ENCODES

Active substance use disorder and documented non-adherence predict graft loss and are contraindications until addressed, since immunosuppression demands strict adherence.

BOARD TRAP

Non-adherence/active addiction is a behavioral contraindication — modifiable; abstinence and adherence support can restore candidacy.

13. Strong functional status favors listing

WHAT YOU SEE

Strong-functional-status admit figure

WHAT IT ENCODES

Adequate functional status and life expectancy to benefit from the graft are required; severe frailty or limited life expectancy may preclude transplant on a risk/benefit basis.

BOARD TRAP

Frailty/limited life expectancy can preclude transplant even with no 'disease' contraindication — benefit must outweigh surgical risk.

14. Boards decision band

WHAT YOU SEE

Bottom band: block / control / absolute-no symbols

WHAT IT ENCODES

Algorithm: ACTIVE infection or malignancy → control first; recent cancer → wait 2–5 yr; positive cytotoxic crossmatch → absolute no-go for THAT donor only; controlled chronic disease/HIV/HCV → eligible.

BOARD TRAP

Single most-tested fact: positive crossmatch = absolute donor-specific contraindication; nearly everything else is relative once controlled.
